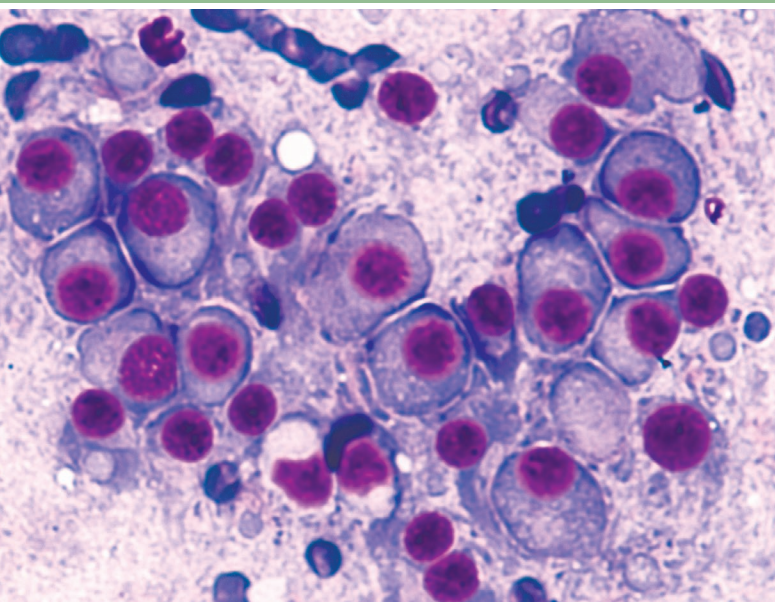


# CancerScope

Oncology Issues in Focus | BY MARY BETH NIERENGARTEN



## NCCN guidelines focus on treatment options for patients with multiple myeloma

Recommendations highlight important updates specific to systemic therapy options for those with newly diagnosed or previously treated multiple myeloma.

**T**he National Comprehensive Cancer Network (NCCN) recently published significant updates specific to treatment options for patients with newly diagnosed multiple myeloma (MM) who are eligible or ineligible for hematopoietic cell transplantation (HCT), recommendations for maintenance therapy, and treatment options for previously treated MM.<sup>1</sup>

Based on the most recent clinical evidence, the recommendations are meant to serve as guidelines, and clinicians are encouraged to use their best judgment when applying the recommendations in the context of individual clinical circumstances.

Ajay K. Nooka, MD, MPH, associate director of clinical research at the Winship Cancer Institute at Emory University in Atlanta, Georgia, points to several new updates for oncologists.

### New definitions for high-risk MM

Based on a collaboration between the International Myeloma Society (IMS) and the International Myeloma Working

Group (IMWG), the guidelines include the IMS–IMWG risk stratification with new definitions of high-risk MM:

- Patients with del(17p) (<20% of plasma cells) and/or a *TP53* mutation.
- Patients with co-occurring translocations with 1q+ and/or a deletion plus t(4:14), t(4:16), or t(14:20).
- Patients with a monoallelic deletion along with 1q+ or a biallelic deletion.
- Patients with high  $\beta$ 2-microglobulin levels (>5.5 mg/dL) and normal creatinine levels (<1.2 mg/dL).

Patients who do not meet the criteria for high risk are considered to be at standard risk.<sup>2</sup>

### Diagnostic workup

New updates for the diagnostic workup of MM include renal biopsy for albuminuria or abnormal renal function and the addition of next-generation sequencing for *TP53* mutation assessment. The updates also include a revised recommendation for baseline clonotype identification at diagnosis and for the storage of an aspirate sample for future clonotype identification to enable minimal residual disease testing through next-generation sequencing.

### Treatment of smoldering myeloma

The update added daratumumab as a high-risk treatment option based on evidence from the phase 3 AQUILA trial showing that patients treated with subcutaneous daratumumab had a significantly lower risk of progression to active MM.<sup>3</sup>

### Treatment of symptomatic MM

The updated recommendations call for the addition of central nervous system disease management, including multimodality therapy, radiation, intrathecal chemotherapy, and systemic chemotherapy, and novel agents such as chimeric antigen receptor (CAR) T-cell therapy.

For primary therapy in patients who are candidates for HCT, the update recommends the quadruple regimen of isatuximab-irfc, bortezomib, lenalidomide, and dexamethasone (Isa-VRd).

For primary therapy in patients for whom HCT is deferred or who are not candidates for HCT, the update recommends either daratumumab, bortezomib, lenalidomide, and dexamethasone or Isa-VRd for patients who are <80 years old and are not frail. Dr Nooka emphasizes that all the treatment recommendations are based on the strongest (category 1) evidence.

### Supportive care

As for supportive care, new infection recommendations for CAR T-cell and bispecific antibody (BsAb) therapies are in the update and include the following:



- For patients receiving levofloxacin, start treatment when the absolute neutrophil count is <500 or per clinician discretion and continue until neutrophil recovery.
- For patients receiving immunoglobulin replacement, continue for the duration of therapy with BsAbs according to the clinical context and for immunoglobulin G levels <400 mg/dL and/or for recurrent life-threatening infections.
- For patients on BsAbs, prophylactic tocilizumab may be considered before the first dose of BsAbs to reduce the risk of cytokine release syndrome.
- For patients on denosumab maintenance, maintenance therapy should be for up to 2 years. Clinical judgment should be used if it is being continued beyond 2 years.

### Treatments useful in certain circumstances

The recommendations provide guidance on the treatment of patients in certain circumstances. Patients can receive more than one B-cell maturation antigen (BCMA)-targeted therapy, although the optimal sequencing of sequential BCMA-targeted therapies is not known. Dr Nooka emphasizes, however, that accumulated data suggest that immediately following with the second BCMA-directed therapy after relapse may be associated with lower response rates.

Other circumstances include the use of belantamab mafodotin-blmf (as available through the expanded-access program) and the use of talquetamab plus teclistamab for patients with MM with extramedullary disease.

### Special populations

To calculate the prognosis of older adults with MM, the update recommends using the Myeloma Frailty Score Calculator developed by the IMWG (<http://www.myelomafrailtyscorecalculator.net/>).

The update recommends more equitable and inclusive clinical trials that include African American and Black people because of the differences in disease biology and responses to treatment found in this population versus White people. African American and Black people have the highest rate of myeloma and differential biology, with most likely to have chromosome 14 translocations (e.g., t(11:14), t(14:16), and t(14:20)) and most less likely to have high-risk mutations (e.g., del(17p) and gain/amp(1q)) compared to White people. Also, the toxicity of drugs can vary in African American and Black people; this includes the hyperpigmentation of skin with the use of immunomodulatory drugs and a higher risk for cytokine release syndromes during CAR T-cell therapy.

### References

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2. Avet-Loiseau H, Davies FE, Samur MK, et al. International Myeloma Society/International Myeloma Working Group consensus recommendations on the definition of high-risk multiple myeloma. *J Clin Oncol*. Published online June 9, 2025. doi:10.1200/JCO-24-01893
3. Dimopoulos MA, Voorhees PM, Schjesvold F, et al. Daratumumab or active monitoring for high-risk smoldering multiple myeloma. *N Engl J Med*. 2025;392(18):1777-1788. doi:10.1056/NEJMoa240902

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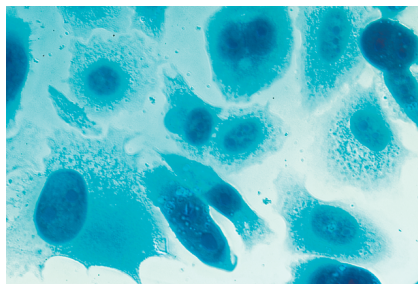
## A persistent PSA level following radical prostatectomy for prostate cancer is associated with a worse prognosis

**W**aiting for at least 3 months to assess the prostate-specific antigen (PSA) level after radical prostatectomy may minimize overtreatment in some patients according to the results of a study published in *JAMA Oncology*.<sup>1</sup>

A conventional assessment of the PSA level typically occurs 1.5–2.0 months after radical prostatectomy, but data on the serum half-life of total PSA after radical prostatectomy suggest that it may take longer for clearance of PSA from the serum, particularly in patients with higher PSA levels before surgery.<sup>2</sup>

Investigators in the current study evaluated how long to monitor PSA after radical prostatectomy to accurately document persistent PSA. They also looked at whether an increasing level of PSA predicts worse outcomes.

Using databases from two academic health centers, investigators identified 43,298 patients with T1N0M0 to T3N0M0 prostate cancer who were treated with radical prostatectomy between 1992 and 2020: 30,461 of the patients



were included in the initial discovery cohort, and 12,837 were included in a subsequent validation cohort.

The study evaluated whether a significant interaction existed between a PSA level greater than 20 ng/mL versus 20 ng/mL or less before radical prostatectomy and a persistent PSA level versus undetectable PSA after radical prostatectomy with respect to prostate cancer-specific mortality (PCSM) risk and all-cause mortality (ACM) risk. The analysis was adjusted for all known prostate cancer prognostic factors.

Investigators found that among patients with a persistent PSA level after radical prostatectomy, a presurgery PSA level greater than 20 ng/mL versus 20 ng/mL or less was significantly associated with reduced ACM risk and PCSM risk. The association remained after adjusting for the prostate gland volume. The PCSM finding was confirmed in the validation cohort.

Investigators said the “counterintuitive” finding may be explained by a higher proportion of patients with a preradical